

NAME:	MS SATPREET KAUR	AGE/SEX:	14Y/FEMALE
REF. BY:	DR AJAY BHUTANI	DATE:	24.05.2018
INVESTIGATION:	MRI BRAIN WITH CONTRAST	S NO:	10185005

STUDY PROTOCOLS:

MR IMAGING OF THE BRAIN WAS PERFORMED USING AXIAL FLAIR, T1 AND T2 WEIGHTED SECTIONS AND CORRELATED WITH T2W SAGITTAL AND FLAIR CORONAL IMAGES, WITH POST CONTRAST IMAGES IN AXIAL, CORONAL AND SAGITTAL PLANES.

FINDINGS:

There is evidence of a relatively well-defined heterogeneously enhancing extra-axial broad dural based mass lesion with CSF cleft is seen in right temporal region, measuring 4x2.6x2.6cm. The mass lesion is appearing isointense on T1W images and heterogeneously hypo-to-isointense on T2W/FLAIR images with surrounding mild perilesional edema is seen.

Remaining cerebral parenchyma shows normal MR morphology with maintained grey-white matter differentiation.

Diffusion weighted imaging carried out does not reveal any area displaying hyperintense signal intensity suggestive of restricted diffusion with increasing 'b' values.

Bilateral ganglio-thalamic regions grossly appear normal in MR morphology.

Ventricular system is normal in size and position with septum in midline.

Sella and parasellar region are normal.

Fourth ventricle is normal in size central in position.

Both the cerebellar hemispheres and brainstem show normal MR morphology.

IMPRESSION: MR study reveals relatively well-defined heterogeneously enhancing extra-axial broad dural based mass lesion with CSF cleft in right temporal region with surrounding mild perilesional edema as described – likely Meningioma.

Advised: Histopathological correlation

Please correlate clinically.


DR Payal Mittal
MD (Radio-Diagnosis)
Formerly PGI (CHANDIGARH)

Disclaimer: The science of radiology is based upon interpretation of shadows of normal and abnormal tissue. This is neither complete nor accurate; hence, findings should always be interpreted in the light of clinico-pathological correlation. This is a professional opinion, not a diagnosis. Not meant for medico legal purposes.

Please intimate us for any typing mistakes and send the report for correction within 7 days.